

Injectons for Pain Conditions



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Medicare Advantage Medical Coverage Policy

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Disclaimer

The Medical Coverage Policies are reviewed by all Utilization Management Committees within Humana. Policies in this document may be modified by a member's coverage document. Clinical policy is not intended to preempt the judgment of the reviewing medical director or dictate to health care providers how to practice medicine. Health care providers are expected to exercise their medical judgment in rendering appropriate care. Identification of selected brand names of devices, tests and procedures in a medical coverage policy is for reference only and is not an endorsement of any one device, test, or procedure over another. Clinical technology is constantly evolving, and we reserve the right to review and update this policy periodically. References to CPT® codes or other sources are for definitional purposes only and do not imply any right to reimbursement or guarantee of claims payment. No part of this publication may be reproduced, stored in a retrieval system or transmitted, in any shape or form or by any means, electronic, mechanical, photocopying or otherwise, without permission from Humana and subsidiaries.

Related Medicare Advantage Medical/Pharmacy Coverage Policies

Treatment/Procedure	Corresponding Medical/Pharmacy Coverage Policy
Hydrodissection; Prolotherapy	Code Compendium (Musculoskeletal and Neurologic)
Occipital nerve block	Headache and Occipital Neuralgia Treatments
Platelet rich plasma (PRP)	Bone Graft Substitutes
Radiofrequency denervation	Neuroablative Techniques for Chronic Pain

Related CMS Documents

Please refer to [CMS Medicare Coverage Database](#) for the most current applicable CMS National Coverage Determination (NCD)/Local Coverage Determination (LCD)/Local Coverage Article (LCA). Refer to CMS website for the most current applicable [CMS Online Manual System \(IOMs\)](#) and [Transmittals](#).

Humana and subsidiaries apply a Medicare Administrative Contractor's (MAC's) LCD to all requests (both Part A and Part B services) within the MAC's applicable geographic jurisdiction.

Type	Title	Document ID Number	Jurisdiction Medicare Administrative Contractors (MACs)	Applicable States/Territories
NCD	Acupuncture for Chronic Lower Back Pain (cLBP)	30.3.3		
	Therapeutic Embolization	20.28		
LCD LCA	Epidural Steroid Injections for Pain Management	L39054 A58777	J5 - Wisconsin Physicians Service Insurance Corporation (Part A/B MAC)	IA, KS, MO, NE
	Facet Joint Interventions for Pain Management	L38841 A58477		
	Nerve Blocks for Peripheral Neuropathy	L35222 A57589	J8 - Wisconsin Physicians Service Insurance Corporation (Part A/B MAC)	IN, MI
	Sacroiliac Joint Injections and Procedures	L39475 A59257		
	Trigger Point Injections	L39713 A59553		
LCD LCA	Epidural Steroid Injections for Pain Management	L39036 A58745	J6 - National Government Services, Inc. (Part A/B MAC)	IL, MN, WI
	Facet Joint Interventions for Pain Management	L35936 A57826		
	Pain Management - Injection of Tendon Sheaths, Ligaments, Ganglion Cysts, Carpal and Tarsal Tunnels	L33622 A52863	JK - National Government Services, Inc. (Part A/B MAC)	CT, MA, ME, NH, NY, RI, VT
	Peripheral Nerve Blocks	L36850 A57452		
	Sacroiliac Joint Injections and Procedures	L39455 A59233		
	Trigger Point Injections (TPI)	L39662		

		A59487		
LCD LCA	Epidural Steroid Injections for Pain Management	L39015 A58731	J15 - CGS Administrators, LLC (Part A/B MAC)	KY, OH
	Facet Joint Interventions for Pain Management	L38773 A58364		
	Intervertebral Disc Repair	L39958 A59880		
	Nerve Blocks for Peripheral Neuropathy	L35249 A57663		
	Sacroiliac Joint Injections and Procedures	L39383 A59154		
	Trigger Point Injections (TPI)	L39656 A59480		
LCD LCA	Epidural Steroid Injections for Pain Management	L39240 A58993	JE - Noridian Healthcare Solutions, LLC (Part A/B MAC)	CA, HI, NV, American Samoa, Guam, Northern Mariana Islands
	Facet Joint Interventions for Pain Management	L38801 A58403		
	Injections - Tendon, Ligament, Ganglion Cyst, Tunnel Syndromes and Morton's Neuroma	L34218 A57079	JF - Noridian Healthcare Solutions, LLC (Part A/B MAC)	AK, AZ, ID, MT, ND, OR, SD, UT, WA, WY
	Intervertebral Disc Repair	L39960 A59882		
	Nerve Blockade for Treatment of Chronic Pain and Neuropathy	L35456 A56034		
	Sacroiliac Joint Injections and Procedures	L39462 A59244		
	Trigger Point Injections (TPI)	L34211 A57701		
LCD LCA	Epidural Steroid Injections for Pain Management	L36920 A56681	JH - Novitas Solutions, Inc. (Part A/B MAC)	AR, CO, LA, MS, NM, OK, TX

	Facet Joint Interventions for Pain Management	L34892 A56670	JL - Novitas Solutions, Inc. (Part A/B MAC)	DE, DC, MD, NJ, PA
	Trigger Point Injections	L35010 A57751		
LCD LCA	Epidural Steroid Injections for Pain Management	L38994 A58695	JJ - Palmetto GBA (Part A/B MAC)	AL, GA, TN
	Facet Joint Interventions for Pain Management	L38765 A58350	JM - Palmetto GBA (Part A/B MAC)	NC, SC, VA, WV
	Intervertebral Disc Repair	L39942 A59866		
	Nerve Blocks and Electrostimulation for Peripheral Neuropathy	L37642 A56731		
	Sacroiliac Joint Injections and Procedures	L39402 A59192		
	Trigger Point Injections (TPI)	L39671 A59498		
LCD LCA	Epidural Steroid Injections for Pain Management	L33906 A56651	JN - First Coast Service Options, Inc. (Part A/B MAC)	FL, PR, US VI
	Facet Joint Interventions for Pain Management	L33930 A57787		
	Injection of Trigger Points	L33912 A57114		
	Peripheral Nerve Blocks	L33933 A57788		

Description

Injectons for pain conditions may be given for either diagnostic or therapeutic (treatment) purposes and may include epidural steroid injections, facet joint injections, regional sympathetic nerve blocks, sacroiliac joint injections, trigger point injections, dry needling of trigger points and/or peripheral nerve blocks. These injections are often included as part of a pain management program.

Epidural Steroid Injections

An epidural steroid injection (ESI) is used to help reduce radicular spinal pain that may be caused by pressure on a spinal nerve root as a result of degenerative disc disease, a herniated disc or spinal stenosis. This treatment is most frequently used for low back pain, though it may also be used for cervical (neck) or thoracic (midback) pain. An anesthetic medication, with or without a steroid (eg, corticosteroid, dexamethasone), is injected into the epidural space near the affected spinal nerve root with the assistance of computed tomography (CT) or fluoroscopy which allows the physician to view the placement of the needle. The goal of this treatment is to reduce inflammation and block the spinal nerve roots to relieve radicular pain or sciatica. It can also provide sufficient pain relief to allow the individual to progress with their rehabilitation program.

Facet Joint Injections

Facet injections, also known as facet blocks or medial branch blocks, are injections of a local anesthetic, with or without a steroid medication, into the facet joints or their nerve supply (the medial branch nerve).

Facet injections may be given for diagnostic purposes to determine if the facet joint is the source of pain² and must be performed under CT- or fluoroscopy-guidance. If the pain is relieved, the physician will know that the facet joint is likely to be the source of pain. A therapeutic facet block or a facet denervation may follow a successful diagnostic facet block.

Regional Sympathetic Nerve Blocks

Regional sympathetic nerve blocks are performed by injecting a local anesthetic into the region of the relevant sympathetic ganglia for the treatment of complex regional pain syndrome (CRPS, previously known as reflex sympathetic dystrophy [RSD]). At the cervical level, these blocks may be referred to as stellate ganglion blocks and in the thoracic or lumbar level as paravertebral sympathetic blocks. As with other blocks, these may both aid in diagnosis of CRPS and be given as a therapeutic injection.

Sacroiliac Joint Injections

Sacroiliac (SI) joint injections are performed by injecting a local anesthetic, with or without a steroid medication, into the SI joint. These injections may be given for diagnostic purposes to determine if the SI joint is the source of the low back pain or may be performed to treat SI joint pain that has previously been diagnosed. If the pain is relieved, the physician will know that the SI joint appears to be the source of pain. This may be followed up with therapeutic injections of anti-inflammatory (steroid) and/or local anesthetic medications to relieve pain for longer periods.

Trigger Point Injections

Trigger point injections (TPI) are injections of a local anesthetic, with or without a steroid medication, into a painful area of a muscle that contains the trigger point. The purpose of a TPI is to relax the area of intense muscle spasm, effectively inactivate the trigger point and provide symptomatic pain relief.

Dry Needling of Trigger Points

Dry needling differs from traditional acupuncture, even though it does make use of acupuncture-type needles. Acupuncture follows the principles of energy flow as a guide to where the needles will be inserted; in dry needling, needles are inserted directly into a myofascial trigger point, in an attempt to inactivate it, thereby theoretically decreasing the associated pain. Dry needling, even though it targets a trigger point, does differ from a trigger point injection, as there is no injection of medication or fluid.

Peripheral Nerve Block

Peripheral nerve blocks consist of injecting a local anesthetic, with or without a steroid, into a peripheral nerve or a nerve ganglion, in an attempt to block pain signals and, in theory, provide prolonged relief from pain. Examples of peripheral nerve blocks include, but may not be limited to, cluneal nerve block, coccygeal nerve block, ganglion impar block, genicular nerve block, obturator nerve block, pudendal nerve block or splanchnic nerve block.

Other Therapeutic Injections

Injectons may also be given into other structures in an attempt to alleviate pain. Examples include, but may not be limited to, iliotibial (IT) band injection, intradiscal injection (also referred to as intervertebral disc repair), pedicle screw block/hardware block of instrumentation used in spinal fusion or sacrococcygeal junction/sacrococcygeal ligament injection.

Genicular artery embolization (GAE) is not an injection in the traditional sense of therapeutic injections (injecting a medication to block a nerve), but is proposed as a treatment for knee pain as a result of osteoarthritis. It is theorized that inflammation in the synovium of the knee joint can lead to vascular endothelial cell proliferation, which promotes hyperplasia and knee vessel inflammation that can contribute to further joint tissue destruction; this may increase chronic pain by facilitating growth of sensory nerves along the newly formed vessels. In GAE, under x-ray imaging guidance, a catheter is advanced to the knee via the femoral artery, and an embolic agent is injected to block the blood flow in the genicular arteries and capillaries supplying the synovium. This purports to reduce inflammation and nerve growth, leading to decreased pain and potential delay of disease progression.

Embolization of arteries other than the genicular artery (eg, those in the ankle, foot, hand, hip, shoulder) is being explored as a potential treatment option for other pain conditions (eg, Achilles tendinopathy, arthritis, plantar fasciitis). These procedures may be referred to as intra-arterial embolization, transarterial embolization, transcatheter arterial embolization or TAE.

Coverage Determination

Humana and subsidiaries follow the Medicare requirements that only allow coverage and payment for services that are reasonable and necessary for the diagnosis and treatment of illness or injury or to improve the functioning of a malformed body member except as specifically allowed by Medicare.

Humana and subsidiaries apply any applicable National Coverage Determination (NCD) and any applicable Local Coverage Determinations (LCDs) to the services and jurisdiction at issue. See the Related CMS Documents Section above for any such NCDs or LCDs, including those related to:

- **Dry needling (NCD 30.3.3 – Acupuncture for Chronic Low Back Pain)**
- **Epidural steroid injections**
- **Facet joint injections/medial branch nerve blocks**
- **Trigger point injections**

In order to determine medical necessity consistently, Humana and subsidiaries may consider the criteria contained in the following:

The following criteria apply to ALL jurisdictions.

Ganglion Impar Blocks

Ganglion impar blocks will be considered medically reasonable and necessary when the following requirements are met:

- Individual with chronic coccydynia or chronic pelvic pain^{57,63}:
 - Failure of conservative treatment (activity modification, ice/heat, pharmacologic, physical therapy)^{57,63}; **AND**
 - Procedure is performed with real-time imaging guidance (CT scan or fluoroscopy) to assure proper needle placement^{57,63} (this is considered ***integral to the primary procedure and not separately reimbursable***⁴²); **AND**
 - Procedure may be repeated ONLY if the prior injection provided at least 50% pain reduction and an improvement in physical function^{57,63};

OR

- Individual with pelvic or perineal pain due to cancer^{52,57,63}:
 - Pain is refractory to pain medications⁵²; **AND**
 - Procedure is performed with real-time imaging guidance (CT scan or fluoroscopy) to assure proper needle placement^{57,63} (this is considered ***integral to the primary procedure and not separately reimbursable***⁴²); **AND**
 - Procedure may be repeated ONLY if the prior injection provided at least 50% pain reduction for 3 months⁵²

The following criteria apply to ALL jurisdictions.

Regional Sympathetic Nerve Blocks (64510, 64520)

Regional sympathetic nerve blocks will be considered medically reasonable and necessary when the following requirements are met⁵¹:

- Diagnosis when sympathetically mediated CRPS is suspected as evidenced by **ALL** of the following criteria being met:
 - Continued, ongoing pain, disproportionate to any inciting event (eg, surgery, trauma); **AND**

- **ONE** or more *symptoms* in **THREE** of the following categories:
 - Sensory: hyperesthesia, allodynia
 - Vasomotor: temperature asymmetry, skin color changes, skin color asymmetry
 - Sudomotor/edema: edema, sweating changes, sweating asymmetry
 - Motor/trophic: decreased range of motion (ROM), motor dysfunction (weakness, tremor, dystonia), trophic changes (hair, nails, skin); **AND**
- **ONE** or more *findings on physical exam* in **TWO** or more of the following categories:
 - Sensory: evidence of hyperalgesia (to pinprick), allodynia (to light touch)
 - Vasomotor: evidence of temperature asymmetry, skin color changes, skin color asymmetry
 - Sudomotor/edema: evidence of edema, sweating changes, sweating asymmetry
 - Motor/trophic: evidence of decreased ROM, motor dysfunction (weakness, tremor, dystonia), trophic changes (hair, nails, skin); **AND**
- Real-time imaging guidance (CT scan, fluoroscopy or ultrasound) must be used to assure proper needle placement for either diagnostic or therapeutic injections^{47,64} (this is considered ***integral to the primary procedure and not separately reimbursable***⁴²); **AND**
- Utilization of these blocks is to be with the intent to allow participation in an active rehabilitation program^{3,51}

Diagnostic Phase:

- A diagnostic block is performed to confirm (or disprove) the presence of sympathetically mediated CRPS⁵¹; **AND**
- If the diagnostic phase is completed and unsuccessful (less than 50% pain relief and no improvement in function), no further injections will be covered⁴⁵

Therapeutic Phase:

- If the diagnostic phase is completed and successful (at least a 50% reduction in pain and improvement in function), therapeutic injections may be initiated⁴⁵;

AND all of the following:

- A 50% reduction in pain is achieved⁴⁵; **AND**

- Decrease in pain medication use³; **AND**
- Improved/increased functional ability (increased ROM, strength and use of the extremity in activities of daily living [ADLs], increased tolerance to touch)³; **AND**
- Ongoing participation in an active rehabilitation program^{3,51}

Sacroiliac Joint Injections (27096)

For jurisdictions without an LCD, Humana and subsidiaries determine medical necessity for **intra-articular sacroiliac joint (SIJ) injections** for the treatment of chronic moderate to severe low back pain based on the criteria contained in **LCD – Sacroiliac Joint Injections and Procedures (L39383)**.

While the CMS guidance for **intra-articular SIJ injections** is well established for most indications, it is **silent regarding blocks to the lateral branch nerve of the SI joint**. Therefore, for ALL jurisdictions, Humana and subsidiaries may consider the following to interpret or supplement such criteria in order to determine medical necessity consistently:

LIMITATIONS:

Sacroiliac joint injections will **not** be considered medically reasonable and necessary for the following indications:

- Lateral branch nerve blocks to the SI joint for diagnostic or therapeutic purposes⁴

The use of the criteria above provides clinical benefits highly likely to outweigh any clinical harms (eg, adverse effects including, but not limited to, inadvertent intra-arterial injection, spinal cord injury^{2,54}). Services that do not meet the criteria above are not medically reasonable and necessary and may result in unnecessary exposure to potential complications. Medically unnecessary services carry risks of adverse outcomes and may interfere with the pursuit of other treatments which have demonstrated efficacy.

Coverage Limitations

[US Government Publishing Office. Electronic code of federal regulations: part 411 – 42 CFR § 411.15 - Particular services excluded from coverage](#)

Intervertebral Disc Repair (Intradiscal Injections) (0627T, 0628T, 0629T, 0630T)

For jurisdictions without an LCD (**J5/J8, J6/JK, JH/JL, JN**), Humana and subsidiaries determine medical necessity for **intervertebral disc repair (intradiscal injections)** based on the criteria contained in **LCD (L39942) – Intervertebral Disc Repair**.

Peripheral Nerve Blocks

For jurisdictions without an LCD (**JH/JL**), Humana and subsidiaries determine medical necessity for **peripheral nerve blocks** based on the criteria contained in **LCD (L36850) – Peripheral Nerve Blocks**.

While LCDs for **peripheral nerve blocks** provide criteria for some indications, they do not provide criteria specific to when the use of **coccygeal, genicular, obturator or pudendal nerve blocks** are considered medically reasonable or necessary. Therefore, Humana and subsidiaries may consider the following to interpret or supplement existing CMS guidance:

The following applies to ALL jurisdictions.

Other Miscellaneous Injections for Pain Conditions

The following **injections** will **not** be considered medically reasonable and necessary for any indication, including for management/treatment of pain:

- Coccygeal nerve block⁶³
- Genicular artery embolization^{50,62}
- Genicular nerve block^{59,66} **(64454)**
- Iliotibial (IT) band injection⁶⁵
- Obturator nerve block⁶⁰
- Paravertebral block for chronic pain⁶¹ **(64461, 64462)**
- Pudendal nerve block⁶⁰ **(64430)**
- Transarterial embolization (also referred to as intra-arterial embolization, TAE, transcatheter arterial embolization) for any musculoskeletal/chronic pain condition^{46,48,49,61,68}

A review of the current medical literature shows that the **evidence is insufficient** to determine that these services are standard medical treatments. There is an absence of current, widely-used treatment guidelines or acceptable clinical literature (as defined by CMS) examining benefit and long-term clinical outcomes establishing the value of these services in clinical management.

The following **injections** will **not** be considered medically reasonable and necessary for any indication, including for management/treatment of pain:

- Pedicle screw block/hardware block of instrumentation used in spinal fusions
- Sacrococcygeal junction/sacroccygeal ligament injection (for any indication, including coccydynia)
- Splanchnic nerve block
- Quadratus lumborum block for chronic pain
- Erector Spinae Plane (ESP) block for chronic pain

- Cervical plexus block for chronic pain

A review of the current medical literature shows that there is **no evidence** to determine that these services are standard medical treatments. There is an absence of current, widely-used treatment guidelines or acceptable clinical literature (as defined by CMS) examining benefit and long-term clinical outcomes establishing the value of these services in clinical management.

Coding Information

Any codes listed on this policy are for informational purposes only. Do not rely on the accuracy and inclusion of specific codes. Inclusion of a code does not guarantee coverage and/or reimbursement for a service or procedure.

CPT® Code(s)	Description	Comments
20550	Injection(s); single tendon sheath, or ligament, aponeurosis (eg, plantar "fascia")	
20551	Injection(s); single tendon origin/insertion	
20552	Injection(s); single or multiple trigger point(s), 1 or 2 muscle(s)	
20553	Injection(s); single or multiple trigger point(s), 3 or more muscles	
20560	Needle insertion(s) without injection(s); 1 or 2 muscle(s)	
20561	Needle insertion(s) without injection(s); 3 or more muscles	
20605	Arthrocentesis, aspiration and/or injection, intermediate joint or bursa (eg, temporomandibular, acromioclavicular, wrist, elbow or ankle, olecranon bursa); without ultrasound guidance	
20999	Unlisted procedure, musculoskeletal system, general	
22899	Unlisted procedure, spine	
27096	Injection procedure for sacroiliac joint, anesthetic/steroid, with image guidance (fluoroscopy or CT) including arthrography when performed	
37242	Vascular embolization or occlusion, inclusive of all radiological supervision and interpretation, intraprocedural roadmapping, and imaging guidance necessary to complete the intervention; arterial, other than hemorrhage or tumor (eg, congenital or acquired arterial malformations, arteriovenous malformations, arteriovenous fistulas, aneurysms, pseudoaneurysms)	
37243	Vascular embolization or occlusion, inclusive of all radiological supervision and interpretation, intraprocedural roadmapping, and imaging guidance necessary to complete the intervention; for tumors, organ ischemia, or infarction	

62320	Injection(s), of diagnostic or therapeutic substance(s) (eg, anesthetic, antispasmodic, opioid, steroid, other solution), not including neurolytic substances, including needle or catheter placement, interlaminar epidural or subarachnoid, cervical or thoracic; without imaging guidance	
62321	Injection(s), of diagnostic or therapeutic substance(s) (eg, anesthetic, antispasmodic, opioid, steroid, other solution), not including neurolytic substances, including needle or catheter placement, interlaminar epidural or subarachnoid, cervical or thoracic; with imaging guidance (ie, fluoroscopy or CT)	
62322	Injection(s), of diagnostic or therapeutic substance(s) (eg, anesthetic, antispasmodic, opioid, steroid, other solution), not including neurolytic substances, including needle or catheter placement, interlaminar epidural or subarachnoid, lumbar or sacral (caudal); without imaging guidance	
62323	Injection(s), of diagnostic or therapeutic substance(s) (eg, anesthetic, antispasmodic, opioid, steroid, other solution), not including neurolytic substances, including needle or catheter placement, interlaminar epidural or subarachnoid, lumbar or sacral (caudal); with imaging guidance (ie, fluoroscopy or CT)	
64430	Injection(s), anesthetic agent(s) and/or steroid; pudendal nerve	
64450	Injection(s), anesthetic agent(s) and/or steroid; other peripheral nerve or branch	
64451	Injection(s), anesthetic agent(s) and/or steroid; nerves innervating the sacroiliac joint, with image guidance (ie, fluoroscopy or computed tomography)	
64454	Injection(s), anesthetic agent(s) and/or steroid; genicular nerve branches, including imaging guidance, when performed	
64461	Paravertebral block (PVB) (paraspinous block), thoracic; single injection site (includes imaging guidance, when performed)	
64479	Injection(s), anesthetic agent(s) and/or steroid; transforaminal epidural, with imaging guidance (fluoroscopy or CT), cervical or thoracic, single level	
64480	Injection(s), anesthetic agent(s) and/or steroid; transforaminal epidural, with imaging guidance (fluoroscopy or CT), cervical or thoracic, each additional level (List separately in addition to code for primary procedure)	
64483	Injection(s), anesthetic agent(s) and/or steroid; transforaminal epidural, with imaging guidance (fluoroscopy or CT), lumbar or sacral, single level	

64484	Injection(s), anesthetic agent(s) and/or steroid; transforaminal epidural, with imaging guidance (fluoroscopy or CT), lumbar or sacral, each additional level (List separately in addition to code for primary procedure)	
64490	Injection(s), diagnostic or therapeutic agent, paravertebral facet (zygapophyseal) joint (or nerves innervating that joint) with image guidance (fluoroscopy or CT), cervical or thoracic; single level	
64491	Injection(s), diagnostic or therapeutic agent, paravertebral facet (zygapophyseal) joint (or nerves innervating that joint) with image guidance (fluoroscopy or CT), cervical or thoracic; second level (List separately in addition to code for primary procedure)	
64492	Injection(s), diagnostic or therapeutic agent, paravertebral facet (zygapophyseal) joint (or nerves innervating that joint) with image guidance (fluoroscopy or CT), cervical or thoracic; third and any additional level(s) (List separately in addition to code for primary procedure)	
64493	Injection(s), diagnostic or therapeutic agent, paravertebral facet (zygapophyseal) joint (or nerves innervating that joint) with image guidance (fluoroscopy or CT), lumbar or sacral; single level	
64494	Injection(s), diagnostic or therapeutic agent, paravertebral facet (zygapophyseal) joint (or nerves innervating that joint) with image guidance (fluoroscopy or CT), lumbar or sacral; second level (List separately in addition to code for primary procedure)	
64495	Injection(s), diagnostic or therapeutic agent, paravertebral facet (zygapophyseal) joint (or nerves innervating that joint) with image guidance (fluoroscopy or CT), lumbar or sacral; third and any additional level(s) (List separately in addition to code for primary procedure)	
64510	Injection, anesthetic agent; stellate ganglion (cervical sympathetic)	
64640	Destruction by neurolytic agent; other peripheral nerve or branch	
64999	Unlisted procedure, nervous system	
76942	Ultrasonic guidance for needle placement (eg, biopsy, aspiration, injection, localization device), imaging supervision and interpretation	

77003	Fluoroscopic guidance and localization of needle or catheter tip for spine or paraspinous diagnostic or therapeutic injection procedures (epidural or subarachnoid) (List separately in addition to code for primary procedure)	
77012	Computed tomography guidance for needle placement (eg, biopsy, aspiration, injection, localization device), radiological supervision and interpretation	
97139	Unlisted therapeutic procedure (specify)	
99152	Moderate sedation services provided by the same physician or other qualified health care professional performing the diagnostic or therapeutic service that the sedation supports, requiring the presence of an independent trained observer to assist in the monitoring of the patient's level of consciousness and physiological status; initial 15 minutes of intraservice time, patient age 5 years or older	
99153	Moderate sedation services provided by the same physician or other qualified health care professional performing the diagnostic or therapeutic service that the sedation supports, requiring the presence of an independent trained observer to assist in the monitoring of the patient's level of consciousness and physiological status; each additional 15 minutes intraservice time (List separately in addition to code for primary service)	
CPT® Category III Code(s)	Description	Comments
0213T	Injection(s), diagnostic or therapeutic agent, paravertebral facet (zygapophyseal) joint (or nerves innervating that joint) with ultrasound guidance, cervical or thoracic; single level	
0214T	Injection(s), diagnostic or therapeutic agent, paravertebral facet (zygapophyseal) joint (or nerves innervating that joint) with ultrasound guidance, cervical or thoracic; second level (List separately in addition to code for primary procedure)	
0215T	Injection(s), diagnostic or therapeutic agent, paravertebral facet (zygapophyseal) joint (or nerves innervating that joint) with ultrasound guidance, cervical or thoracic; third and any additional level(s) (List separately in addition to code for primary procedure)	
0216T	Injection(s), diagnostic or therapeutic agent, paravertebral facet (zygapophyseal) joint (or nerves innervating that joint) with ultrasound guidance, lumbar or sacral; single level	

0217T	Injection(s), diagnostic or therapeutic agent, paravertebral facet (zygapophyseal) joint (or nerves innervating that joint) with ultrasound guidance, lumbar or sacral; second level (List separately in addition to code for primary procedure)	
0218T	Injection(s), diagnostic or therapeutic agent, paravertebral facet (zygapophyseal) joint (or nerves innervating that joint) with ultrasound guidance, lumbar or sacral; third and any additional level(s) (List separately in addition to code for primary procedure)	
0627T	Percutaneous injection of allogeneic cellular and/or tissue-based product, intervertebral disc, unilateral or bilateral injection, with fluoroscopic guidance, lumbar; first level	
0628T	Percutaneous injection of allogeneic cellular and/or tissue-based product, intervertebral disc, unilateral or bilateral injection, with fluoroscopic guidance, lumbar; each additional level (List separately in addition to code for primary procedure)	
0629T	Percutaneous injection of allogeneic cellular and/or tissue-based product, intervertebral disc, unilateral or bilateral injection, with CT guidance, lumbar; first level	
0630T	Percutaneous injection of allogeneic cellular and/or tissue-based product, intervertebral disc, unilateral or bilateral injection, with CT guidance, lumbar; each additional level (List separately in addition to code for primary procedure)	
HCCPS Code(s)	Description	Comments
No code(s) identified		

Summary of Evidence

Ganglion Impar Block

An evidence-based clinical resource⁵⁷ found some published evidence suggests that safety and impact on health outcomes are at least comparable to standard treatment/testing. The authors identified 6 comparative and 3 noncomparative studies evaluating the use of ganglion impar block for the treatment of chronic coccydynia. Of those studies, 2 were considered fair quality studies that compared the use of ganglion impar block to alternative treatments including the use of local anesthetic alone and caudal epidural steroid injections. A third fair quality study randomized individuals undergoing ganglion impar block to 1 of 2 separate approaches to the treatment of ganglion impar. Findings suggest that ganglion impar blocks for the treatment of chronic coccydynia is likely associated with clinically and statistically significant reductions in pain for at least 3 months after treatment.

Regional Sympathetic Nerve Blocks

Chen et al⁴⁵ performed a retrospective cohort study including 318 individuals who underwent regional sympathetic nerve blocks for the treatment of Complex Regional Pain Syndrome (CRPS) and other sympathetically related disorders. Successful pain reduction was noted in 61% of the individuals who were being treated for CRPS. Additionally, 85% of those individuals experienced more than 50% pain relief for 1-4 weeks or longer. Chen et al⁴⁵ conclude that regional sympathetic nerve blocks provide clinically significant pain reduction in a majority of patients with CRPS or other sympathetically related disorders.

Lateral Branch Nerve Block

The American Society of Regional Anesthesia and Pain Medicine (ASRA)⁴ concludes that the use of lateral branch nerve block for diagnostic or therapeutic management of sacroiliac joint pain is a less attractive approach compared to intra-articular sacroiliac joint blocks due to the failure to block or innervating the anterior joint, the variable location of the lateral branch of the sacral roots, and the risk of spread of the anesthetic into other pain-generating structures.

Coccygeal Nerve Block

An evidence-based clinical resource⁶³ gave a weak recommendation based on low quality evidence for use of coccygeal injections containing local anesthetic or local anesthetic plus glucocorticoid in patients with persistent coccydynia of greater than 2 months.

Genicular Artery Embolization for Chronic Pain

In a systematic review and meta-analysis for osteoarthritis-related knee pain treated with genicular artery embolization (GAE), Torkian et al⁶² found the quality of the studies to be fair in 8 and poor in 3 (according to the National Institute of Health quality assessment tool), as well as a relatively small sample size (n = 225 patients; 268 knees). General limitations of the studies include a lack of a control group (utilizing other treatment modalities), as well as short duration of follow-up. The authors conclude that due to the paucity of high-quality trials, further research is warranted to evaluate GAE's long-term outcomes, its comparative efficacy with other treatment modalities, and its role in the therapeutic approach of this condition and patient population.

In another systematic review, Taslakian et al⁵⁰ report outcomes for genicular artery embolization for treatment of knee osteoarthritis pain. This meta-analysis investigates the safety and effectiveness of the procedure. A total of 270 patients (339 knees) were included in the studies, and patient follow up ranged from 1 to 48 months, with 12 months being the median. One unique finding reported in this study is the postulation that patients with greater knee pain severity may be more responsive to GAE, though a reason for this was not offered and notation includes that this should be hypothesis-generating only, and warrants further study. The authors report several limitations, including inconsistent details regarding the GAE treatment, inconsistent definition for technical success, included studies were primarily case series with limited sample sizes, all leading to a conclusion that the comparative safety and effectiveness of GAE relative to other knee OA treatments remains unclear.

Genicular Artery Embolization for Hemoarthrosis

In a systemic review for the use of geniculate artery embolization (GAE) for the treatment of post-total knee arthroplasty related hemarthrosis, Voltreas et al⁶⁹ found 20 studies (9 case reports and 11 case series) with a total sample size of 212 (n = 212). Of those cases, procedure success was 94.8% (n = 200/211). Symptomatic improvement was only seen in 72.6% of those cases (n = 119/164) with 30.7% (n = 58/189) requiring repeat embolization. Recurrent hemarthrosis occurred in 22.2% (n = 22/99) of the cases over a mean follow up of 48 months. Voltreas et al⁶⁹, concluded that GAE for the treatment of post-total knee arthroplasty related hemarthrosis appears to be a safe and effective treatment, but future studies in the form of randomized trials should be conducted to compare embolization against standard treatment.

In a systemic review and meta-analysis for the use of selective artery embolization for the treatment of hemarthrosis following total knee arthroplasty, Nadeem et al⁵³ found 15 studies including 303 participants. The clinical success rate across these studies was 76%, while the recurrence rate for hemarthrosis was 25%. Key limitations to these studies include publication bias, lack of larger prospective trials, and lack of long-term follow-up. While Nadeem et al⁵³ concludes that selective artery embolization is a safe and potentially effective treatment for hemarthrosis following total knee arthroplasty, it is noted that there is a need for larger prospective studies to better define long-term efficacy and safety.

Genicular Nerve Block

An overall very low-quality body of evidence suggests that genicular nerve blocks combined with corticosteroid appears safe, but findings are inconsistent as to whether genicular nerve blocks results in significant improvements in pain and function compared to baseline. Uncertainty remains due to conflicting evidence and limited follow-up to determine long-term or comparative outcomes.⁵⁹

Genicular nerve block has been suggested as an option for individuals who have failed other treatments and for those for whom surgery is not an option. However, the experience for this treatment is still limited and is not widely available. The authors note that meta-analyses show improvements in pain in the short-term, but the certainty of evidence is very low to moderate and data on the long-term outcomes are scarce.⁶⁶

Iliotibial (IT) Band Injection

There is a small quantity of high-quality evidence available regarding treatment of iliotibial band syndrome (ITBS). A limitation in treating ITBS is uncertainty regarding the origin of the pain and whether the cause is due to a tendon injury or inflammation. The recommended treatment from an evidence-based clinical resource⁶⁵ relies mostly on case series, expert opinions and clinical experience; that recommendation is glucocorticoid injections for patients with persistent significant pain despite being compliant with 6 to 12 weeks of acute and subacute treatment. Limited evidence suggests that glucocorticoid injections provide short-term relief of pain caused by ITBS; however long-term benefits are questionable.

Obturator Nerve Block

An independent technology assessment organization⁶⁰ reports that there currently is not enough published peer-reviewed literature to evaluate nerve blockade of the articular branches of the femoral and obturator nerves for treatment of hip arthritis in a full assessment. The authors identify one very small retrospective clinical study (n=20), with short follow-up of only 3 months.

Paravertebral Block for Chronic Pain

An independent technology assessment organization⁶¹ note a lack of evidence regarding the efficacy of paravertebral blocks (PVBs) when used for chronic pain. The authors also note that while PVBs may be associated with a decrease in short-term pain following breast surgery, it is unclear whether long-term outcomes are also improved.

Pudendal Nerve Block

An independent technology assessment organization⁶⁰ review of literature found several concerning limitations to published studies. Only 6 studies evaluate the efficacy and safety of pudendal nerve block for pudendal neuralgia (n=13 to 82) including 1 randomized controlled trial, 4 prospective uncontrolled studies, and 1 retrospective uncontrolled study. The studies were small and used different inclusion criteria, different imaging methods and differing agents for injection. None of the studies were placebo-controlled. Most studies were limited to 1 center or a single surgeon. The organization concludes that the overall quality of the available evidence is very low, and is insufficient to support the routine clinical use of pudendal block for treatment of pudendal neuralgia.

Transarterial Embolization

Ciaffi et al⁴⁶ performed a systematic review and meta-analysis of minimally invasive interventional procedures for osteoarthritis and inflammatory arthritis, which includes use of transcatheter arterial embolization (TAE). The authors conclude that while minimally invasive interventional procedures may improve pain and functional status in those patients, further research is warranted. Risk for bias includes relatively small numbers of participants as well as short follow-up. The authors note challenges in synthesizing evidence across diverse studies and the complexity of accounting variability in study populations, interventions and outcomes.

Park et al⁴⁸ report on a pilot study reviewing clinical outcomes of transarterial embolization for chronic Achilles tendinopathy that has been refractory to traditional conservative treatment. This small (n=20) retrospective study did not report any serious adverse events during follow-up, and reports findings that TAE may alleviate pain, showing a decrease of at least 50% of reported pain at 6 months in 86% of procedures. The authors, however, note several limitations, including lack of a control group, the small number of participants and continued use of conservative treatments after the TAE if the patient's pain persisted (which may have biased clinical outcomes).

Sasaki et al⁴⁹ reviewed study results of ultrasound-guided intra-arterial embolization targeting abnormal neovessels for treatment of plantar fasciitis. This retrospective study followed a small number of participants (n=66) for a mean duration of 30.9 months, finding that this procedure may be effective for plantar fasciitis. The authors concede that the mechanism underlying the therapeutic effect remains unclear, while also noting no significant adverse events with the procedure. The final conclusion notes that while treatment may decrease pain and normalize tendon tissue, further studies with a control group are warranted. The authors also note several limitations, including retrospective studies which possibly make it easier to follow patients for whom treatment was effective, no control group was included and the study was conducted using a small cohort. An evidence-based clinical resource⁶⁸ also notes two case series have reported that transcatheter artery embolization may provide significant pain relief in those with refractory plantar fasciitis, but placebo-controlled trials are required to confirm these findings.⁶¹

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Change Summary

01/01/2024 New Policy.
 12/19/2023 Update, No Coverage Change.
 03/26/2024 Update, Coverage Change.
 07/01/2024 Update, No Coverage Change.
 09/10/2024 Annual Review, Coverage Change. Updated Coding Information
 12/10/2024 Update, No Coverage Change
 10/01/2025 Annual Review, Coverage Change. Updated Coding Information
 10/14/2025 Update, No Coverage Change.
 02/02/2026 Update, No Coverage Change.
 08/03/2026 Provider Claims Codes Update, No Coverage Change.
 09/01/2026 Annual Review, Coverage Change. Updated Coding Information